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Trauma-Informed Perspectives: A Case Study Approach to Understanding and Addressing Trauma

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Abstract :

Trauma has been a focus of scientific research and clinical treatment for decades; nonetheless, its definitions, diagnostic criteria, and societal attitudes frequently remain misconstrued. This systematic meta-analysis offers a trauma-informed examination of the history, clinical definitions, and diagnostic applications of trauma, with the objective of elucidating myths that impede effective care and comprehension. The paper examines the historical development of trauma psychology, starting with its foundations in psychodynamic theory and advancing to modern frameworks like the DSM-5-TR and SAMHSA's "Three E's" of trauma. Misrepresentations of trauma in popular discourse and social media, along with systemic deficiencies in trauma-informed practices, are rigorously analyzed to highlight the pervasive impact of trauma on individuals and social institutions. Essential findings underscore the significance of survival reactions, the neurobiological effects of trauma, and the necessity of implementing trauma-informed care (TIC) to avert re-traumatization and facilitate recovery. The study advocates for a sophisticated comprehension of trauma, highlighting the necessity for accuracy in diagnostic standards and customized interventions for varied populations. This meta-analysis addresses clinical and cultural barriers, thereby expanding the field of trauma psychology and improving global mental health outcomes.

Keywords: Trauma-Informed Care (TIC), Post-Traumatic Stress Disorder (PTSD), Diagnostic and Statistical Manual of Mental Disorders (DSM), Emotional Dysregulation, Adverse Childhood Experiences (ACEs), Trauma Recovery, Trauma Psychology

Introduction:

A Concise History & Examination of Trauma:

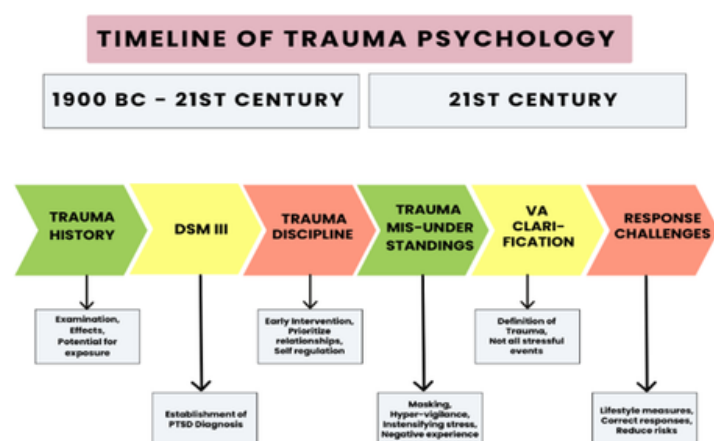
The third edition of the Diagnostic and Statistical Manual of Mental Disorders (DSM-III) established the diagnosis of post-traumatic stress disorder in 1980 (Gold, 2017 b.). In 2006, the American Psychological Association officially acknowledged trauma as a scientific discipline & established Division 56, Division of Trauma Psychology (Gold, 2017, a.). Although some regard trauma psychology as contemporary

and nascent field of study, its historical examination and discussions concerning its essence and effects can be traced back to 1900 BC, persisting to the present regarding the potential for exposure to induce detrimental behavioural and psychological disorders. (Gold, 2017, a.; Van der Kolk, 2014). The failure to acknowledge symptoms resulting from trauma exposure and unaddressed psychological injuries continues to be evident among first responders, judicial

judicial systems, medical and psychological practitioners, educational institutions, and communications from traditional and social media (Gold, 2017, b.; Huo, et al., 2023; Substance Abuse and Mental Health Services Administration [SAMHSA], 2014 b.). The concept of trauma has increasingly become intertwined with ambiguity, permeating contemporary discourse and often being trivialized through memes on social media platforms.

Figure 1:

Timeline of Trauma Psychology: Historical Milestones and Contemporary Challenges :



It is often employed in popular culture as hyperbole to describe generalized difficulties, which distracts from clinical diagnosis and diminishes the complexity and severity of the psychological effects on individuals affected by trauma (Gold, 2017, a.; Maté & Maté, 2022; McFillin, 2024). The Department of Veteran Affairs (2023) elucidated that the term trauma frequently denotes several forms of stressful events and emphasized that not all stressful situations lead to the emergence of post-traumatic stress and anxiety-related disorders. A prevailing consensus highlights that misunderstandings regarding the definition of trauma and the inappropriate use of diagnostic criteria hinder the acknowledgment of its complexity and extensive effects on individuals and social systems. Misunderstandings regarding trauma create deficiencies in understanding how to take preventative measures, respond correctly, avoid re-traumatizing affected individuals, and reduce the risks of secondary and vicarious traumatization when delivering services (Gold, 2017 b.; McFillin, 2024; SAMHSA, 2014 b.)

Trauma: Clinical Definition and Diagnostic Utilization :

Originating from the psychodynamic tradition, Sigmund Freud (1920) and Anna Freud (1964) characterized trauma

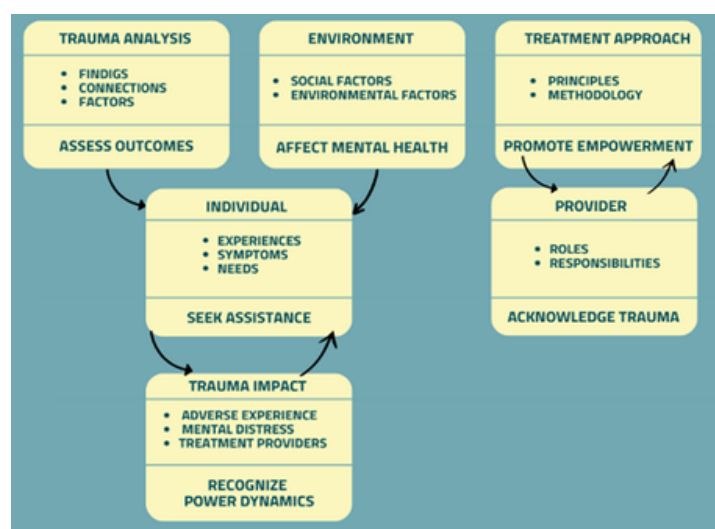
as a "stimulation that surpasses an individual's capacity to cope," resulting in a rupture within both inter- and intrapsychic experiences (Gold, 2017, p. 15). Maté & Maté (2022) clearly articulated their definition of trauma as an internalized injury stemming from an event, emphasizing that trauma is not merely an external occurrence but rather an internal response to traumatic event(s). This implies that individuals and groups may encounter one or several objectively acknowledged traumatic events without necessarily becoming traumatized as a consequence of that exposure. Instances of trauma encompass natural disasters, warfare, physical or emotional abuse, domestic and interpersonal violence, and the illness or death of a loved one (SAMHSA, 2014 a). The effect of a traumatic experience is contingent upon the individual(s) affected by the event(s) (Maté & Maté, 2022). SAMHSA (2014 b.) compiled and reviewed decades of research to create an inventory of terminology and contacted a panel of clinical practitioners, researchers, and trauma survivors to formulate a precise clinical definition. The panel identified intricate commonalities and minor differences that underscored the intricacies of trauma's effects on people, families, groups, and communities (Gold, 2017, a; SAMHSA, 2014, b).

Research findings (SAMHSA, 2014, a., p. 7) indicate that trauma can impact individuals of all races, ethnicities, ages, sexual orientations, psychosocial backgrounds, and geographic regions, with no one being innately immune. The Department of Veteran Affairs (2023) indicated that most individuals would encounter at least one traumatic event in their lifetime, and responding to such an event is seen as a normal reaction, frequently resulting in complete remission. The Substance Abuse and Mental Health Services Administration (SAMHSA, 2014 b., p. 8) defines trauma-informed care through the "Three E's" of Trauma: Event(s), Experience of Event(s), and Effect. Events denote circumstances involving actual or imminent threats of physical or psychological harm, or severe life-threatening neglect that jeopardizes a child's healthy development. Experience of Events emphasizes the individual's subjective interpretation and significance attributed to the events, aiding in the determination of whether the events were traumatizing. The resultant effects of trauma are essential to evaluate, as they may manifest as either immediate or delayed short-term or long-term symptoms (SAMHSA, 2014 b.). The DSM-5-TR (2022) emphasizes the Three "E's" and explicitly requires exposure to traumatic or stressful Event(s) as a diagnostic criterion (SAMHSA, 2014 b.). The DSM-5-TR (2022, p. 295) advises practitioners to exercise caution in diagnosing, as the array of traumatic disorders may manifest as either singular or comorbid diagnoses, encompassing "reactive attachment disorder, disinhibited social engagement disorder, posttraumatic stress disorder,

acute stress disorder, adjustment disorders, and prolonged grief disorder.” The DSM-5-TR emphasized that symptoms causing psychological distress from exposure to trauma-inducing events are diverse and may present in complex, combined, or mixed forms, potentially encompassing anxiety and fear responses, characteristics of anhedonia and dysphoria, dissociation, or externalized anger and aggression.

Figure 2:

Framework for Understanding & Addressing Trauma: Key Components & Processes:



The U.S. Department of Veterans Affairs (2023) predicted that 6% of the U.S. population, or 6 out of 100 individuals, will experience PTSD and attain full recovery during their lifetime, while at least 5% of the adult population may encounter PTSD in any given year. In 2020, over 13 million individuals in the U.S. were likely experiencing PTSD. The example offered by the Department of Veterans Affairs (2023) effectively demonstrated the pervasive and enduring effects of trauma. The World Health Organization [WHO] (2024) estimates that almost 70% of the global population may encounter a stressful experience, with only 5.6% likely to acquire PTSD. The WHO (2024) indicated that PTSD rates are 15.6% elevated in individuals exposed to violent conflict and war, particularly following experiences of sexual assault. The WHO (2024) indicated that PTSD rates are 15.6% elevated in individuals exposed to violent conflict and war, particularly following experiences of sexual assault.

Trauma-Driven Behavior: A Systemic Analysis:

An individual's trauma can affect others through both direct and indirect exposure, resulting in secondary and vicarious consequences (Gold, 2017, c.).

Exposure to trauma induces disorder in affected individuals, and a crucial aspect of comprehending trauma is the recognition and examination of the inter- and intrapsychic dynamics (Bailey, 2021; SAMHSA, 2014 a.). Undergoing a traumatic incident and its subsequent imprint will compel people and social groups to adopt a survival mentality to address threats to their wellness or life (Maté & Maté, 2022; van der Kolk, 2014). Survival stances originate from the cognitive functions of the limbic system, regarded as the basic region of the brain that governs secondary functions (Bailey, 2021; van der Kolk, 2014). The phrases Flight, Fight, Freeze, and Fawn pertain to survival responses (SAMHSA, 2014 a.). Survival stances are coping techniques triggered in response to perceived threats, characterized by ingrained behavioural patterns especially developed for protection and survival (Bailey, 2021; Van der Kolk, 2014). It is essential to acknowledge that traumatic responses are not located in the rational mind; instead, trauma imprints itself inside the limbic system, immediately affecting the body's physical functioning and manifesting as emotional dysregulation exhibited behaviourally (Van der Kolk, 2014). Threats may be explicit or implicit, conveyed verbally or inferred from nonverbal cues and observable behaviour. Consequently, responses to threats against personal safety or that of loved ones, as well as risks to livelihood, relationships, and physical or emotional well-being, may appear irrational to individuals who fail to identify the symptoms. Trauma induces emotional dysregulation, negatively affecting marital dynamics (Bailey, 2017; Morrison & Ferris, 2009; van der Kolk, 2014). Neurobiology indicates that trauma affects the complete triune of the mind and significantly influences general behaviour and emotional responses, which can be reflected in bodily functions (Bailey, 2021; Van der Kolk, 2014). Adverse childhood experiences, including sexual abuse and assault, emotional and psychological abuse, neglect, hunger, and physical trauma, can lead to cognitive impairment. SAMHSA (2014) indicated that this should not suggest that individuals experiencing trauma, especially catastrophic events, are beyond treatment or potential recovery (SAMHSA, 2014; Van der Kolk, 2014).

Methods:

Fundamental Elements of the Trauma Informed Care Framework:

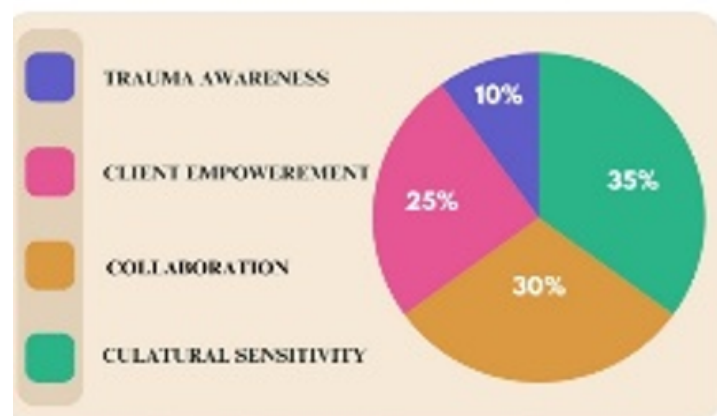
Sweeney and Taggart (2018) elucidated that research on traumatic childhood experiences significantly contributed to the formulation of Trauma-Informed Care (TIC). The analysis conducted by Sweeney and Taggart (2018) revealed that individuals traumatized by their experiences exhibited poor outcomes and demonstrated a significant correlation between the frequency and variety of adverse experiences as

causal factors in the emergence of mental distress (Sweeney & Taggart, 2018, p. 383). Additionally, it established connections between childhood trauma and adult psychosis, depression, interpersonal violence, self-harm, suicidal ideation and attempts, as well as premature mortality. Social and environmental factors, such as poverty and racism, were identified as contributing elements to the emergence of anxiety and stress-related disorders among socially disadvantaged populations (Sweeney & Taggart, 2018). Three principal causes necessitated the development of the TIC approach. The initial findings were shaped by statistical data from surveys conducted in the United Kingdom and the United States, which revealed the significant number of individuals who have experienced trauma and underscored the prevalence of those seeking medical and mental health services due to its adverse effects (Sweeney & Taggart, 2018). To more effectively meet the population's requirements, practitioners should enhance client connections by adopting a trauma-informed approach and collaborating to reduce symptoms. Three principal causes necessitated the development of the TIC approach. The initial findings were shaped by statistical data from surveys conducted in the United Kingdom and the United States, which revealed the significant number of individuals who have experienced trauma and underscored

Figure 3:

Key Elements and Their Proportions in Trauma-Informed Care:

KEY ELEMENTS IN TRAUMA INFORMED CARE



The prevalence of those seeking medical and mental health services due to its adverse effects (Sweeney & Taggart, 2018). To more effectively meet the population's requirements, practitioners should enhance client connections by adopting a trauma-informed approach and collaborating to reduce symptoms. The second aspect recognized that treatment providers may inadvertently exacerbate an individual's tension and anxiety through conventional procedural practices, leading to re-traumatization.

It is crucial to recognize that several individuals may be susceptible to intrusive re-experiencing of trauma, as this is a symptom of Acute Stress Disorder under Criterion B for Post-Traumatic Stress Disorder (Substance Abuse and Mental Health Services Administration [SAMHSA, 2014 a.]). First responders, clinical practitioners, service providers, and communities could gain from recognizing the power dynamics and perceptions held by their clients; for instance, adopting an expert stance may appear dismissive of the client's experiences and convey a "power-over" dynamic (Sweeney & Taggart, 2018; Van der Kolk, 2014). The SAMHSA delineates four "R's," which are essential assumptions that members of a trauma-informed care (TIC) organization must embody: a fundamental realization of trauma and its impact on individuals, families, communities, groups, and organizations; the ability to recognize the signs of trauma; a proactive commitment to respond by integrating and applying trauma-informed principles across all organizational domains, including behaviour and language, ensuring adherence to established policies; and a collective effort to resist re-traumatization (SAMHSA, 2014 a., SAMHSA, 2014 b.; Gold, 2017, c.). Trauma is increasingly recognized in medical and psychiatric disciplines, as well as in society (Bailey, 2021; Gold, 2017 b.; 2017 c.; Mcfillin, 2024; SAMHSA, 2014 a) Trauma is increasingly recognized in medical and psychiatric disciplines, as well as in society (Bailey, 2021; Gold, 2017 b.; 2017 c.; Mcfillin, 2024; SAMHSA, 2014 a). Therapies are becoming increasingly efficient and effective. Nonetheless, further progress is required in comprehending the intricacies of trauma and its capacity to, directly and indirectly, affect individuals, as well as to propagate among interpersonal units, groups, and communities, alongside identifying the optimal care strategies (Gold, 2017 c.). Holistic well-being ought to be recognized by medical and psychiatric professionals. For trauma-informed care to be effective, understanding trauma and acquiring knowledge are essential elements. The TIC approach strives to be acutely attuned to this dimension of care for clients, service providers, and communities (KCMHS, 2013, SAMHSA, 2014).

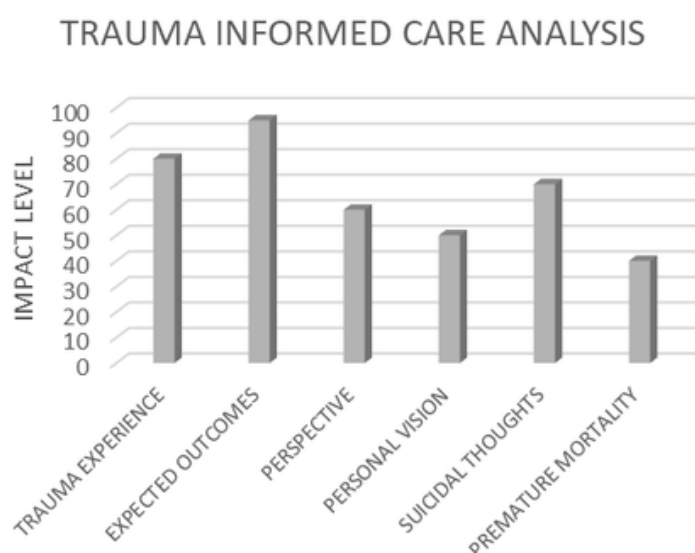
Trauma-Informed Systemic Care: A Focus on Recovery-Oriented Approaches:

The probability of encountering a traumatic incident is significant, and many individuals will suitably react with a stress response, exhibit symptoms, and attain complete recovery (U.S. Department of Veterans Affairs, 2023; WHO, 2024). While providing consolation, this does not diminish the experiences of people affected by trauma, as well as the consequent mental health and drug use disorders and fatalities (SAMHSA, 2014 a.). Recovery and symptom alleviation have been noted in children and adults who received robust social support during or shortly

incident (Sweeney & Taggart, 2018). Nonetheless, the indicators of trauma are not always apparent to the untrained observer. Certain individuals experience symptoms and remain undiagnosed due to their inability to identify these symptoms and challenges in obtaining suitable medical and psychological care (U.S. Department of Veterans Affairs, Sweeney & Taggart, 2018; WHO, 2024). SAMHSA (2014, a.) advocates for organizations to adopt a trauma-informed approach to deliver prevention services and rehabilitation, thereby reducing the transmission of emotional contagion, secondary trauma, vicarious trauma, and preventing re-traumatization.

Figure 3:

The Efficacy of Trauma-Informed Approaches in Reducing Secondary Impacts: A Quantitative Analysis:

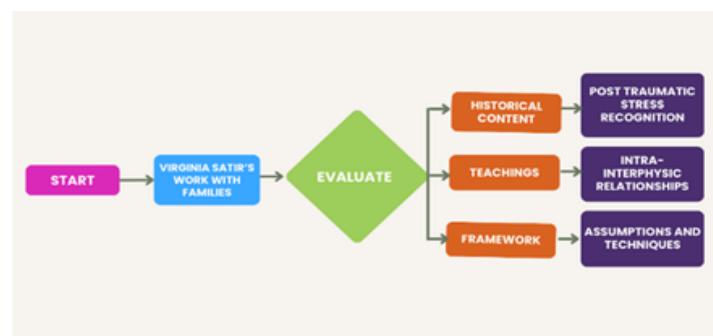


TIC employs a systemic methodology aimed at delivering localized precision in addressing many forms of trauma, including natural disasters, immigration challenges, and violent assaults by unidentified perpetrators (KCMHC, 2013; SAMHSA, 2014 b.; Van der Kolk, 2014). The six principles of Trauma-Informed Care (TIC) encompass: 1.) Safety; 2.) Trustworthiness and Transparency; 3.) Peer Support; 4.) Collaboration and Mutuality; 5.) Empowerment, Voice, and Choice; and 6.) Recognition of Cultural, Historical, and Gender issues (SAMHSA, 2014 a). TIC facilitates the establishment of peer support within a community through active engagement and heightened awareness, transcending mere observation. TIC actively seeks to restore the environment and ensure its safety (SAMHSA, 2014).

Satir's Developmental Framework:

Figure 4:

Therapeutic Process Based on Satir's Assumptions:



Marriage and family therapists view the family unit as a dynamic entity interconnected with other emotional units, creating behavioural patterns to maintain balance or homeostasis in functioning. Satir elucidated that a family's system can be analyzed through the stress responses of its members. Family members encountering stress or exposure to traumatic events will adopt survival strategies to manage the stress or threat, resulting in immediate or protracted emotional dysregulation (Bailey, 2023). Persistent emotional dysregulation within the family system fosters the emergence of anxiety and stress-related symptoms and disorders, establishing a transgenerational pattern (Banmen, 2002). Satir identified five communication attitudes, referred to as survival stances: Super reasonable, Irrelevant, Leveling, Placating, and Blaming (Banmen, 2002). Satir's communication stances resemble the Flight, Fight, Freeze, and Fawn survival and stress responses (van der Kolk, 2014), and neurobiological imaging has revealed a correlation between all these phenomena (Bailiey, 2021). Survival stances are automatic reactions to conflict, intended to safeguard our existence (Bailey, 2021; Banmen, 2002; Morrison & Ferris, 2009; SAMHSA, 2014a; van der Kolk, 2014). Nonetheless, when extended dysregulation transpires, survival strategies may become maladaptive after the threat has subsided.

Results:

Critiques of the Trauma-Informed Approach

McFillin (2024) contended that the expansion of the trauma definition led to an overgeneralization of the term, so oversimplifying the intricate complexities of human behavioural and emotional experiences.

Moreover, Mcfillin (2024) emphasized several legitimate apprehensions among first responders and clinical practitioners, including the perils of self-diagnosis stemming from misconceptions derived from a generalized understanding of trauma and marriage and family systems theory, which has led many to identify and justify their behaviours through a lens of victimhood, thereby undermining accountability, empowerment, and resilience essential to the recovery process. McFillin (2024) recognized the presumption that TIC had been portrayed as a universal solution to intricate intra- and interpsychic experiences that intersect with environmental and relational dynamics throughout individual and transgenerational lifespans. A consensus exists among numerous clinicians. In 1964, Anna Freud first expressed her worries regarding the excessive use of the phrase (Gold, 2017, b.). Prominent psychologists and trauma treatment professionals Gabor Maté, Bessel van der Kolk, and Peter Levine agree (Maté & Maté, 2022). McFillin's (2024) observations were perceptive and emphasized the necessity for more attention to post-traumatic growth. Nonetheless, one could contend that, despite persistent misconceptions, the clinical definition of trauma, the manifestation of symptoms, and the capacity for both direct and indirect emotional and physical contagion—transmitted from individuals to families and communities—result in a transgenerational pattern stemming from traumatic imprints that are unequivocal. Trauma-informed therapeutic practitioners employ evaluations, genograms, scales, and measurements to establish or exclude an accurate diagnosis and to track the maintenance or reduction of symptoms (APA, 2022; Bailey, 2021; Banmen, 2002; SAMHSA, 2014 a.). Moreover, the concerns articulated by Mcfillin (2024) appear to substantiate the necessity for individuals to adopt a more trauma-informed approach. Consequently, misconceptions may diminish, and traumatization would no longer encounter invalidation or trivialization.

Conclusion:

Misconceptions regarding trauma remain with minimal opportunity for refutation. Trauma is intrinsically both intricate and straightforward. Trauma exposure may manifest as a complicated disorder that might masquerade as various anxiety and stress-related disorders (APA, 2022). Although exposure to trauma is nearly inevitable, not all individuals have negative effects (Sweeney & Taggart, 2018; SAMHSA, 2014, a.; WHO, 2024). Recovery is attainable for individuals who exhibit symptoms of a trauma-related condition. Technology, Information, and Communication (TIC) combined with a systemic humanistic framework like Satir's Growth Model are effective for recovery (Bailey, 2021; SAMHSA, 2014; WHO, 2024). As a survivor and a trauma-informed systemic marriage and family therapist, I assert, with inherent bias, that post-traumatic growth is indeed a valuable and achievable objective.

Conflict of Interest :

The author reports no conflict of interest regarding this article.

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